

CHAPTER 6

The Role of the Therapist in IFS

The relationships we therapists develop with clients are governed by our feelings and beliefs. If we believe that people have the ability to deal effectively with their problems, we focus on discovering and changing whatever constrains their inborn abilities. If, on the other hand, we believe that people have problems because they lack something—whether that something is a strong ego, a workable understanding of the problem, a nurturing parent or mate, skills training, or chemistry—then we try to give clients what they lack with interpretations, information, teaching, directives, reframes, reparenting, or drugs. These constitute two very different approaches with clients. One is a collaborative, democratic partnership, which conveys confidence that clients have what it takes to be self-reliant and in relationship, the other is more of a hierarchical relationship in which clients are seen to be lacking or broken.

In practice, most therapists vacillate between these approaches. The collaborative therapist sometimes gives information or offers advice and the therapist who embraces the role of the authority figure encourages clients to use their own resources at times. The difference is one of emphasis over the course of minute-to-minute interactions and the therapist's overall view of human motivation. The IFS model asserts that we all have the inner resources we need. We assume that parts are constrained by systemic imbalances and burdens, and we believe that the client has the necessary resources (the Self) to release them and harmonize the inner system. Since IFS views the relationship between the client's Self and the client's parts as the primary healing force, the therapist's main job is to help the client access enough Self. As Hermann Hesse (1927/1975) wrote, "I wanted only to try to live in accord with my true Self. Why was that so very difficult?" The IFS model aims to help people overcome obstacles so they can live in accord with their true Selves.

Although the big conceptual shift in IFS involves our focus on the healing power of an internal relationship, the external therapeutic relationship remains crucial because protectors enter therapy on high alert regarding vulnerability, trust, and exposure. Before they will let the client access her Self, most protectors need to feel that the therapist is safe, trustworthy, confident, and compassionate. They need to know the therapist.

TRANSFERENCE AND COUNTERTRANSFERENCE

From an IFS perspective, parts who respond to the present as if it were the past are frozen in the past. Psychodynamic therapies refer to this response as *transference* and *countertransference*. Clients sometimes view a therapist as if she were someone from their past (transference) and, conversely, the therapist sometimes views a client as one of his parents or someone else important (countertransference). The IFS model has its own way of understanding and handling this phenomenon. We do not think the client is seeing the therapist as a parent, sibling, or abuser; we think that a part of the client is seeing the therapist that way—a part who is frozen in the past and burdened by extreme beliefs or feelings that developed at a time when something challenging happened with that person. We call this transference reactivity a *trailhead*—meaning an opportunity to discover more about experiences that have affected the client negatively.

TRAILHEADS

Any time we notice a feeling, thought, or physical sensation, that is a *trailhead*. A trailhead might, for example, involve a female client noticing that she feels desperate for her male therapist's approval. In IFS, noticing this feeling of desperation—the trailhead—is an opportunity to get to know (and help) a part who felt rejected by her father and who continues to have a big influence on the client. Meanwhile, let's say the client also has a part who sees the therapist as untrustworthy and pushes him away because the part expects the therapist to reject her the way her father did. Although the therapist could talk to the client about this feeling of longing and the urge to push him away as transference feelings, he can also skip forward to asking if the parts who long for redemption and those who want to push him away would like to get help from the client's Self, which doesn't hold these anachronistic views and feelings. Once the client's Self retrieves and unburdens that rejected child (the longing part), the pushing away part will be released from its job. In this way, we use transference (and countertransference) feelings to locate parts who need help, just as we use any strong feeling or belief.

WHEN THE CLIENT'S SELF IS UNAVAILABLE

Clients are particularly reliant on the therapeutic relationship when they cannot access their Selves quickly, which is often the case for people who have experienced severe, chronic abuse and have strong protective systems that keep the Self hidden or out of the body. Although this stalemate around accessing the Self is usually temporary, it impedes co-therapy between the client's Self and the therapist's Self. Therefore the therapist's Self has to step in and speak directly to the client's parts until the client's Self is available. We call this intervention *direct access*.

Although direct access is an invaluable option, it exposes the therapist to the transferred expectations and accusations of extreme parts. As a result, the therapist who uses direct access can feel besieged and can be challenged to maintain Self-leadership. This challenge is made a lot easier when we keep in mind that we are dealing with a young part who is frozen in another time. Our Self-energy will help the target part see the difference between hurtful people from the past and us. And while the therapist's Self may need to be a primary attachment figure for the clients' parts for a while, this is only until the client's parts are willing to let the client's Self take the lead.

COLLABORATION

The IFS therapist trusts clients to see their predicaments in useful ways and act effectively once constraints (burdens) have been released. Therefore, we rarely offer interpretations, reframes, or directives regarding problems. Our expertise lies not in figuring anything out, but in validating and exploring the client's system with an eye to making space for the client's Self. By communicating the assumptions of IFS and asking questions, we aim to release constraining polarizations and, ultimately, burdens.

In IFS the client's needs determine the balance of focus between inside and out. If you are a beginner, it can be particularly helpful to notice (and help) any of your protectors who cling to safe and familiar practices that keep an external focus. At the same time, the client may be reluctant to focus inside, and you may need to use his language (it's a feeling, a thought, etc.) for a while as you notice his parts. In any case, regardless of our language and the client's willingness, we are doing IFS anytime we keep parts in mind and relate to the client from our Self. Enduring curiosity about (and memory for) the details of the client's life fosters a healing connection. Our explicit support for the risks the client takes, our acceptance of her perceived failures and the parts she has come to believe are shameful, our collaborative strategizing about how to handle difficult relationships and

predicaments, and our excitement about her successes are all important ways of keeping a client company on her journey. At the same time, because any part can short-circuit a Self-led therapeutic relationship, we are open about our parts, which means we speak up and take responsibility when our parts get in the way.

Collaboration means we join the client (and, when appropriate, the client's family) to identify parts and release constraints on Self-leadership. We also spell out IFS assumptions: Everyone has parts; parts live in a web of relationships, much like a family; parts can end up in constraining roles and need help; all parts are valuable; and everyone has, at their core, a Self with the ability to lead. We are curious, compassionate, and respectful partners in therapy. Although we take a leadership role initially by asking questions about certain kinds of constraints and suggesting ways of unblending parts, once the client's Self emerges and takes the initiative, we immediately cede leadership. And when we do make suggestions or lead the client through parts work, we always respect the client's Self's expertise on his experience and solicit his ideas about interventions.

Thus, primary responsibility for change is not placed on the therapist as it is in some therapies, nor is it placed on the client as it is in others. Instead, in IFS the Selves of client and therapist act as co-therapists, sharing responsibility. They collaborate to harmonize the client's inner system and relationship to the external world. In the case of fearful, resistant protectors, the co-therapy of Selves in client and therapist simply develops more gradually, as the therapist earns their trust over time. This collaborative stance offers the IFS client the opportunity to feel cared for and accompanied throughout therapy.

Once a Self-led co-therapy partnership is established, clients often do inner work between sessions, sometimes following through on plans that were devised in a session, sometimes exploring further on their own. Clients' ability to listen and be available to their parts outside of the therapist's office empowers them and shortens the length of therapy. At the same time, the therapist's role in IFS requires far less toil and takes less of a toll than therapeutic models that call on the therapist to offer insight and direction to people who have lost their compasses. Since the primary attachment figure for the client is the client's Self, we take the principles of attachment theory inside. In our experience, this can be a profound shift for the process and outcome of psychotherapy.

THE CLIENT'S CORRECTIVE EXPERIENCE IN IFS

The experience of expecting to be rejected and humiliated but feeling accepted and valued instead is in itself emotionally corrective. In IFS tenacious caring helps clients welcome all their parts—a

radical stance since this includes parts who have taken ugly, costly steps to try to protect the client at the expense of others. Of these parts we say, *This part is not what it appears to be, nor is it who you are. It, too, needs your help.* As our clients access their Selves, they are empowered by experiencing the goodness and strength of who they really are.

Beatriz Looks Back on Therapy

Beatriz, a lesbian of Latinx descent who had experienced various forms and intensities of bias and bullying throughout her life, had come to therapy for the first time at the age of 73 because her 16-year-old granddaughter, who was also gay, wanted to race motorcycles. Two years later, as she cut back to occasional appointments, she told her therapist about her first impressions.

BEATRIZ: You know what I said to myself when I saw you that first time? God, he looks like one of those hippies who hitchhiked to New Mexico and got old there. How could he possibly understand my life? He'll be a voyeur and I'll pay the bill!

THERAPIST: That's your lookout part? He was keeping you alert.

BEATRIZ: Yeah.

THERAPIST: I'm glad he let you stay to check me out.

BEATRIZ: You know what? He liked your eyes. They seemed kind. And when I told you I was an old dyke who didn't want her granddaughter to die young, you just replied, *I know what it's like to worry about children.* You didn't seem shocked or afraid.

THERAPIST: But as I recall you had a part who reacted strongly when I said, *Would you like to talk to the part who is afraid for your granddaughter?*

BEATRIZ: Yeah! I thought, *Here we go!*

THERAPIST: You thought I was saying you were mentally ill?

BEATRIZ: Oh yeah! A brown-skinned lesbian walks in the room and wow! She's like Sybil. I think I was pretty rude to you at that point.

THERAPIST: I guess I should have explained more before using that language. I'm sorry.

BEATRIZ: Even though my lookout guy was sure you were pigeonholing me, another part said, *No! Let's listen for a minute. This seems different.* And then I got interested when you explained your perspective. I was afraid of you, but you didn't seem to be afraid of me, even when I got mad sometimes. You spoke right to my angry part in a friendly way, and said, *Don't take my word for anything.*

THERAPIST: So you stayed.

BEATRIZ: I did. But I had a part who was worried about being disappointed by you for a long time.

THERAPIST: I remember!

In their first exchange, the therapist had communicated curiosity, calm, and confidence. He had presented a new way of looking at the mind to Beatriz's wary inner system. And he had respected the need of her protectors to make sure his approach would be safe. The sense of safety in IFS comes, first and foremost, from the therapist conveying confidence and curiosity, and having a validating rather than pathological vision of how the client got to where she is today. We also communicate patience and respect for "resistance," which we view as the valid need of protective parts to ensure that the client is safe. Because therapy is a parallel process for therapist and client, our patience, perspective, and presence are contagious.

WHEN A THERAPIST'S PART GETS IN THE DRIVER'S SEAT

Since the key to maintaining a collaborative relationship with clients is leading from the Self, we monitor our parts and keep them from getting in the driver's seat during therapy. We need to stay aware of parts who get activated by the role of psychotherapist as well as parts who react to certain types of clients or problems. Many therapists can achieve enough Self-leadership to be effective with certain populations, especially those with whom they feel comfortable, without having ongoing therapy. But to become intuitive with the IFS model there is no substitute for getting to know your inner family. Our parts—the parts of therapists—often interfere with our effectiveness. When a client behaves like one of your exiles or one of your less popular protectors, staying Self-led can be a challenge. If, for example, you fear being with a vulnerable exile of your own, you may have the urge to distract when the client's vulnerable exile emerges. Or if you have parts who are frustrated with your inner critic, you are at risk of being impatient with the client's inner critic.

Many clients have been shamed as children and then rejected as too sensitive, emotional, or impulsive when they reacted to the shaming. Many have never heard an apology. Those who have experienced severe abuse, betrayal, and neglect may be prone to overreacting because their protectors automatically see their partners (or therapists) as perpetrators. As a result, their relational lives are fraught. When we are Self-led we offer our most injured clients the opportunity to be close to someone who, seeing the pain underlying their abrasive behavior, can rebalance internally and remain a compassionate presence (Schwartz, 2013).

BURDENED THERAPISTS

As clients get closer to their exiles and become more vulnerable, extreme protectors often push back, which in turn can provoke the therapist's protectors to do harm. Our Self-leadership is most crucial when the client is most vulnerable. When we hear negative thoughts about the client in our head or notice a judgmental tone to our voice, we know that our system needs help. When we ask our protectors to step back and promise them that we will check in later, our return to Self-leadership may be enough to help the client's protectors relax as well. On the other hand, we can also use these moments to model speaking for parts by saying something like, "I just noticed one of my judgmental protectors reacting to your critic. Did you notice it, too? I've asked it to step back. I apologize."

Staying Self-led can certainly be a challenge at times. Extreme parts in clients often evoke strong emotions in therapists, at which point our managers are more likely to jump in and pathologize the clients. When our managers take over we are at risk of becoming either professionally detached, defensive, and controlling, or dropping appropriate boundaries and trying to rescue clients. Rescuing, in turn, is likely to stir up an inner polarity between our caretaking parts and parts who resent having so much responsibility for others. This is when therapists start calling clients manipulative. When we close our hearts, our clients feel abandoned and their protectors activate. We know of many cases in which a client decompensated and ended up in the hospital overmedicated because a therapist was unaware of his own reactivity. While some of these clients responded quickly to the IFS perspective, got better, and were able to reengage in therapy, others were far slower to trust a therapist again.

Although minimizing or ignoring the influence of our parts is bad medicine, we do not mean to imply that therapists can expect to engage in this deeply intimate process without their parts activating. When a client shows weakness or neediness, tells us we are dead wrong, or challenges our competence, our unhealed wounds put us at risk of not doing our job well. And who among us has not been wounded and then suffered the ill effects of our own inner protection? Who has gone without the faithful company of an inner critic? Who has not hated their weakness, neediness, or anger? And, conversely, who has not felt angry with those external others who seem to judge weakness or neediness?

Clients can step on the landmines of any number of common vulnerabilities and polarizations within their therapists in all innocence, perhaps simply looking like a feared member of the therapist's family of origin; perhaps quarreling with a spouse the way the therapist's parents quarreled; or perhaps acting out the way the therapist did as a teenager, which the therapist's vigilant managers continue to judge harshly. When a client activates our polarized protectors and we deliver back-to-back contradictory messages without noticing, we are the opposite of therapeutic. We need to be

aware that our protectors are entirely capable of confusing, injuring, and defeating our clients. This is why in IFS we focus on staying Self-led in the presence of our clients' protectors. We meet the challenge of maintaining Self-leadership by noticing our parts during sessions, and by depolarizing our protectors and unburdening our exiles outside of sessions.

COMMON BEHAVIORS OF THERAPISTS' MANAGER PARTS

Managers in therapists are interested in the same things as all managers: controlling appearances, performances, and relationships. They live by a “never again” managerial philosophy. *I will never let you get so close to anyone again, I will insist that you perform perfectly and look perfect, I will never let you drop your guard in any way.* Managers are vigilant, hardworking, and preemptive in their attempts to keep exiles out of mind. They criticize. They try to prevent us from feeling too much, or perhaps from feeling anything at all. They keep us in our heads. They remind us of past failures. They numb the body. They inhibit. They control. They also carry their own burdens. Managers often believe, for example, that we have something vile hidden inside that would repulse anyone else who saw it, that we are too defective to succeed so we shouldn't try, that we have to be perfect or no one will like us, and that we truly are unlovable. They can be very young. And they get frozen in traumatic scenes, often believing that we, too, are that younger age. Following is a list of common behaviors that therapists' managers display during therapy sessions:

1. *Striving managers*: Grow critical if change isn't quick enough. Because they cannot bear weakness and vulnerability, they can be highly directive or coercive. They say things like:
 - “You have to tell her what this is all about. She's too overwhelmed to understand.”
 - “He's been coming to see you for over a year—some brief therapist you are!”
 - “All she does is cry—why can't she quit sniveling and move on?”
2. *Approval-seeking managers*: Often want clients to depend on or worship a therapist. These parts worry about not being liked or seen as effective if a client is displeased or upset.
 - “Now you've done it! She's mad at you. She doesn't like you. She's going to tell everyone that you and your stupid therapy model are no good.”
3. *Pessimistic managers*: Might tell a therapist to give up, or might blame either the therapist or client (or both) when things aren't going well. Ironically, these parts are often trying to keep us from continuing and either feeling disappointed or being disappointing.
 - “He's much sicker than you thought.”
 - “You don't know what you're doing.”

- “She’s so manipulative, why should you care about her pain?”
- 4. *Caregiving managers*: Need to rescue and can’t stand being with clients who are upset.
 - “You have to do it for him—he’s obviously incapable.”
 - “You are a bad therapist if you let her suffer.”
- 5. *Angry managers*: Cause the therapist to feel impatient and burdened.
 - “What does he want now? It’s always some big crisis.”
 - “She’s so dependent and demanding—why can’t she be stronger?”
 - “Maybe he’ll cancel this week.”
- 6. *Evaluating managers*: Criticize the therapist’s weight, eating habits, or indulgences, and criticize the same in others.
 - “My God, she’s thinner than you and she’s complaining about her weight. What a pig you are!”
- 7. *Sensitive, fearful managers*: Overidentify with a client’s pain.
 - “How can he stand talking about that? It was too horrible! Don’t listen, this is too painful.”
 - “She can’t take any more. Do something! Get her to stop now.”
 - “You make such an effort to steer clear of all this. This is really too much like your experience. Just find him a new therapist.”

Although we need not and cannot always be a model of Self-leadership, we can model taking responsibility for our parts. We have an axiom in IFS. When we encounter a problem in therapy, it means that a part is probably interfering—but we don’t know whose part it is: the client’s or ours. IFS therapists practice being aware of and helping their own parts. We either help our parts to relax and trust our Self in the moment, or, when that’s not happening despite our best efforts, we acknowledge to the client that a part is interfering and apologize, and work with our part later.

BEING THE “I” OF THE STORM

Compassionate Self-leadership is often easier to prescribe than to accomplish on a consistent basis. Therapy is full of predictable yet unanticipated ups and downs. A therapist who is invested in alleviating symptoms quickly will feel happy when a client is doing well but discouraged, defensive, and pessimistic when she is not doing well—which will only feed the client’s pessimism and inner recriminations. The therapist who fears anger or distancing will be challenged to maintain Self-

leadership in the face of angry or distancing parts of the client. The therapist who dislikes anger and disdain will struggle with disliking and reacting defensively to members of the client's family who are angry or disdainful. Our clients help us find the trailheads that lead to our exiles.

If the therapist can unblend from reactive parts and remain steady, curious, and confident while sailing turbulent waters—in short, can be the “I” of the storm—then the client and her family will be helped to tolerate relapses and be curious about obstacles that arise. The therapist and client who can be curious together in the face of extremes can explore setbacks calmly while taking steps to repair and prevent recurrences. When we aim to learn from relapses, we fear them less.

There are many potential landmines in IFS therapy. In the face of provocation, even constant provocation, the Self-led therapist expresses what might be called *tenacious caring*, which is caring that helps the therapy succeed against the odds. The rewards for working this way are as great for the therapist as for the client because efforts to maintain Self-leadership are extremely therapeutic. In this way, our most difficult clients become our most useful tormentors. By tormenting us they mentor us, helping us to find the parts we need to heal.

CONCLUSION

Therapists who maintain at least some degree of Self-leadership will, by definition, feel and express curiosity and compassion for their clients' predicaments and respect for their clients' abilities. This does not mean that therapists will not feel other emotions like anger, happiness, or sadness. We don't banish our parts. They are with us and color our experiences. Clients may appreciate feedback from our nonextreme parts, and IFS therapists are encouraged to report to clients on their parts when appropriate. But before we speak for one of our parts we must, if possible, determine whether its perceptions are distorted. If we cannot make this determination, we should be honest and report our uncertainty to the client.

We are responsible for keeping our parts from distorting and interfering during sessions, for being in good communication with our parts outside of sessions, and for leading with the compassion and respect inherent to Self-leadership, which is conveyed through tone of voice as well as nonverbal behaviors. When we lead from the Self, we can challenge or confront clients and still communicate respect and compassion. While we have so far emphasized the therapist as collaborator, there are times when clients are too blended with extreme parts to be aware of them. This is particularly true of parts who carry our culture's legacy burdens of racism, patriarchy, materialism, and individualism (which we cover in [Chapter 18](#)) because we have marinated in those juices since we were children

and the beliefs these parts hold can either be an implicit bias or can simply feel like a self-evident truth.

As well, many clients have parts who deny or minimize the impact of their addictions, narcissism, grandiosity, or privilege. Consequently, there are times when the therapist needs to say some version of, “I get that you don’t see it, but a part of you is wreaking havoc in your relationships, and we need to be in better communication with it. I know it is trying to protect you and it isn’t bad. We can help it out of its protective role if you’re willing.” Clients are generally less resistant to Self-led confrontations like this than they would be to traditional confrontations for a couple of reasons. First, it’s not about them—this is just a part and that part is acting protectively so it’s not bad; and, second, the therapist’s tone and demeanor convey caring and confidence that the part can change its behavior. Clients tend to stay open when they feel that we care about them, we are concerned, and we want to help. Like a tuning fork, our Self elicits their Selves.